

Letter of instruction — psychiatric report

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From: Cory Lea Shepherd · WC/2024/227, Queensland Industrial Relations Commission · [DATE]

■ **DRAFT.** Three bracketed items require confirmation before sending.

PART A — THE ATTACHMENTS

Four documents accompany this letter. **Three of them were written by the other parties to my matter.** That is deliberate, and it is the point of Part B.

No.	Document	Author	Status
1	Response to Notice to Admit Facts , 18 February 2026 — the Workers' Compensation Regulator's admissions in this appeal.	The Regulator (the respondent)	Assumed fact
2	Letter of the Chief Executive, Metro South Hospital and Health Service , 5 June 2026 (ref K-LM26/729), to the Commission.	The employer's Chief Executive	Assumed fact
3	Role description — Administration Officer, Switchboard Services (AO3) , Logan Hospital.	The employer	Assumed fact
4	The clinical record — your own file for me from 24 October 2024; my general-practice records (Our Medical Ashmore), including the entries of 26 October 2022 and 16 November 2023; the Work Capacity Certificate of Dr Hawes; and my certificate of capacity of 3 July 2026.	Clinicians	Clinical record

PART B — THE BASIS ON WHICH I ASK YOU TO WRITE

I ask you to assume, for the purpose of your opinion, **only** the facts recorded in Attachments 1 to 3. I do not ask you to assume anything on my account of the workplace.

Attachment 1 records facts admitted by the respondent in the proceeding. Attachments 2 and 3 are the employer's own documents. Where your own history from me differs from any of those facts, **please say so expressly** rather than resolve the difference. Attachment 4 is the clinical record, provided for your clinical review and not as facts about the workplace.

Please do not adopt, or treat as established, any conclusion, characterisation or finding of any other decision-maker. **Your opinion is yours alone.** The legal questions in the appeal are not asked of you, and I do not ask you to express any view on whether any management action was reasonable — that is a question for the Commission.

What the appeal is about, in two sentences. I appeal the rejection of a claim for a psychological injury with onset on 18 June 2024. The matters in issue concern rostering and fatigue, remuneration, and the handling of concerns I raised; it is **not** a case of bullying or harassment, and I ask that the report describe the matter accordingly.

PART C — DIAGNOSIS, CAUSATION AND MECHANISM

1. Diagnosis, criteria and onset. The diagnosis; the classificatory framework applied and the code; the specific diagnostic criteria met and when each was met; and the date of onset as best the clinical record establishes it.

2. Differential diagnoses. Those considered, and your reasons for including or excluding each.

3. Psychiatric background. Please address the entries of 26 October 2022 referring to anxiety and ADHD, any earlier stimulant use, and the entry of 16 November 2023. Whether there was any relevant pre-existing condition; and if so, whether the matters at Attachments 1 to 3 aggravated it, and to what extent.

4. Sources. A complete list of every document you reviewed and every person you saw, with the dates and duration of each examination.

5. The exposure. Attachment 3 records that the position requires continuous shift work across the full 24-hour period, seven days a week; participation in the Emergency Response process, distributing emergency notifications in accordance with code procedures and strictly adhering to protocols and timeframes; maintaining call queues to a minimum at all times; multitasking and operating under pressure with high volume call traffic; exercising judgement where precedent has not been set and procedures are not defined; and working with limited supervision. Attachment 1 records that the accuracy of Switchboard information is critical to clinical handover and patient safety. Attachment 2 records that no fatigue risk assessment applied to the position, that fatigue risk management was implemented at the Switchboard only after 30 June 2024, that complaints were managed solely by email or verbally, and that no consequential changes to operating procedures were made over the period.

From a clinical perspective, whether and how conditions of that kind — the level of demand, the degree of control, the adequacy of support, and fatigue arising from the rostering — can contribute to a condition of the kind you have diagnosed.

6. The rostering of 17 to 18 March 2024. Attachment 1 records a rostered break of 7 hours against a 10-hour minimum, described as a rostering error accidentally made. Attachment 2 records that emergency (MET) calls were in fact recorded on those shifts, and that leave was taken on 19 March 2024. Please state what clinical significance, if any, you attach to that sequence.

7. Causation. On the assumed facts, whether my employment was **a significant contributing factor** to the injury with onset on 18 June 2024. Please give your clinical reasoning, and please distinguish between the cause of onset and any factors bearing only on the subsequent course of the condition.

8. The interval. The rostering matters at question 6 occurred in March 2024 and onset is pleaded at 18 June 2024. From a clinical perspective, is that interval consistent with, or inconsistent with, your formulation — and if consistent, by what mechanism? Separately, what significance, if any, do you attach to the interval between onset and first presentation, the Work Capacity Certificate recording first attendance for this injury on 1 July 2024?

9. Competing causes, placed in time. Please identify any non-employment factors relevant to (a) onset at or about 18 June 2024, and (b) the subsequent course. Your report of 13 February 2025 referred to multiple life stressors including relationship breakdown, job loss and bereavement.

Your clinical relationship with me began on 24 October 2024. Each of those matters therefore arose while I was under your care and is recorded in your own file. I ask you to place each factor in time **from your own records rather than by assumption**, and to state, for each, its significance to (a) the cause of onset and (b) the subsequent course.

[CONFIRM: the events of 24 October 2024 and what, precisely, is to be said about support from that date — Cory to settle before sending.]

10. Premorbid personality. Whether any premorbid personality features bear on the diagnosis and on causation, and if so how — from your own current clinical assessment. I do not ask you to adopt or repeat any earlier characterisation.

PART D — CURRENT CAPACITY, RESTRICTIONS AND ADJUSTMENTS

My employer has asked me for medical information about my capacity. I ask you to address the following so that this report answers those matters as well. My certificate of capacity of 3 July 2026 certifies me fit for my substantive role with adjustments; I ask only that your opinion be your own.

11. Prognosis and current capacity. Your prognosis; and, distinguishing (a) my capacity to perform the substantive role **with reasonable adjustments** from (b) any current incapacity. If there is current incapacity, its cause — the condition itself, the consequences of my exclusion from the workplace since 3 July 2026, or other factors.

12. The adjustments, in functional terms. Please specify the adjustments you consider clinically necessary, the clinical basis for each, and the anticipated duration and review date. Please address, among anything else you consider relevant: predictability of rostering; adequate recovery between shifts; sustained concurrent emergency-code load; hours of work; and reporting arrangements.

13. Inherent requirements. Whether, **with the adjustments at question 12**, I am able to fulfil the requirements of the position described at Attachment 3; and whether those adjustments are ordinary and available in a workplace of that kind.

14. Exacerbating conditions. Whether there are specific tasks, situations or environments that may exacerbate the condition or its symptoms.

15. Complaint handling. If you consider a restriction on complaint handling clinically necessary, please state what activities it encompasses in functional terms — receiving, documenting, redirecting or resolving complaints, or all complaint-related interaction.

16. Working memory. Your report of 13 February 2025 records that my working memory is affected under stress. Please clarify what that means in functional terms in a workplace, including how it may affect performance of the duties at Attachment 3 and the circumstances likely to give rise to the difficulty.

17. Foreseeable risk. Whether exposure to the conditions described at Attachments 2 and 3, without the adjustments at question 12, presents a foreseeable risk to my health or safety; and if so, what controls you consider medically necessary to manage it.

18. Questions I do not ask of you. I do not ask you to express a view on whether my employer is able to accommodate the adjustments you recommend, or on my capacity to work under any particular reporting line or with any particular person. Those are workplace questions, not medical ones. If you are asked either question, I ask that you say so.

PART E — THE FORM OF THE REPORT

Please reason from the assumed facts to your conclusions rather than assert conclusions. Please state where the material is insufficient for you to express an opinion, rather than qualifying an opinion you would not otherwise give.

Please state expressly the material on which your opinion as to causation rests, and whether that opinion depends on my account of the workplace matters.

The report is prepared for use in proceedings in the Queensland Industrial Relations Commission and may be provided to my employer. [CONFIRM: the standard practice footer restricting use must not appear on this report.]

[CONFIRM: fee, and whether Review Decision 69983 was provided at the consultation of 12 August 2026 — if it was, an express direction not to adopt any finding in it, and a question whether the opinion would differ if it were disregarded, must be added.]